

The significance of past history in homœopathic prescribing*

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It is a privilege to speak in memory of one of the great homœopaths, a man who brought a scholarly mind and immense energy to the practice of Homœopathy. Richard Hughes greatly admired Hahnemann but never hesitated to disagree when careful assessment of the facts led him to other conclusions. He disbelieved Hahnemann's theory of chronic disease. Hughes wrote:¹ "I would again impress this fact upon such of his disciples as represent Homœopathy to be a complete scheme of medical philosophy: who would make the dynamic origin of all maladies a plank of the platform on which we must stand and call the psora-hypothesis the homœopathic doctrine of chronic disease. This is an entire mistake. There are certain views in physiology and pathology which seem more harmonious than others with homœopathic practice. Hahnemann thus came to hold them and most of us tend in the same direction. But they might all be disproved and abandoned and Homœopathy would still remain the same. We should still examine patients and prove drugs and administer remedies on the same principles and with the same success."

Hughes practised low potency pathological prescribing which was usual in Britain until shortly before the first world war when Sir John Weir and others brought over the teachings of James Tyler Kent and other great American homœopaths. Kent's philosophy, representing his interpretation of Hahnemann's theory of chronic disease, is the main guide to homœopathic practice in Britain today. While the general trend is along Kentian lines, there has, however, been a tendency to look at his philosophy more critically, to regard his teaching as an immensely valuable guide, rather than as representing absolute truth. Tyler, for instance, suggested the possibility that psora might consist of the aftermath of one or more of any infections, acute or chronic, other than sycosis and syphilis, rather than a single infection, which most of us have never seen or at any rate recognized.

CONSTITUTIONAL TREATMENT

Constitutional homœopathic treatment consists mainly of prescribing on the psychosomatic make-up of the patient and on the past history. The patient is viewed against a background of a theoretical average person of the same age, sex and social environment, and the salient mental and physical characteristics are noted and matched by a remedy having a similar drug picture. One or more, usually a number of remedies carefully chosen in this way can frequently restore health when there are no insuperable pathological barriers. Quite often the

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psychosomatically selected remedy can cover outstanding episodes in the past, for example *Nat. mur.* when the patient has had concussion, but sometimes it seems that it is necessary directly to antidote some past event which greatly disturbed the patient's health. The patient's past history and the family history are similarly viewed against the background of a theoretical average family and personal history and outstanding features or events are noted. Then, if psychosomatic prescribing fails to produce satisfactory results, the past history may lead to an appropriate remedy for that individual, or it may be advisable to prescribe on the family or individual history when there are no clear symptomatic indications for any one remedy right from the beginning.

OBSERVATIONS ON THE HOMŒOPATHIC MATERIA MEDICA

Failure to achieve results is usually put down to incorrect prescribing, very often quite rightly, but it should be appreciated that the homœopathic materia medica is incomplete. There are no records of any unsuccessful proving in over a century and a half, which suggests that all, or nearly all, substances on earth are capable of affecting man in the potentized if not in the crude state; which means that there are several thousand potential homœopathic remedies as yet untouched. Even if the almost insuperable difficulties of proving and clinically testing were overcome, the addition of so many new medicines would tend to offset any gains by making the already unwieldy and complex materia medica so large that it would be virtually impossible for anyone to become familiar with it and the difficulties of remedy selection would be even greater than they are at present. The immense field covered by the existing materia medica is surprising, however the selection was made. The only way that appreciable progress can be made through provings is to be able to predetermine the potential value of the substance to be proved. However difficult the problem, much more thought must be given to it.² Most recent provings have been almost entirely wasted because there is little incentive to carry out an intensive clinical trial of a remedy which might in the end turn out to be of very little value, although the provings themselves have been carried out scientifically and with the utmost care. Clinical testing is necessary to provide the more important part of the drug picture.

In the meantime, obviously the best possible use should be made of the valuable materia medica available, and careful attention to past history is one way by which this can be achieved. Also, in view of the limitations of the proved and tested materia medica, there should be no hesitation in prescribing unproved remedies such as the nosodes of acute disease as occasion demands.

This is not intended to imply that the vast resources of the homœopathic materia medica of thoroughly tested remedies should be in any way neglected or that the exacting discipline of remedy selection on a symptomatic basis can be escaped. The object of this paper is rather to draw attention to the value *sometimes* obtainable from the patient's past history, and to justify the administration of unproved potentized medicines in certain specified circumstances.

NOT WELL SINCE—

Chronic ill-health may follow a severe acute or chronic infection, injury, emotional upset or, occasionally, after adverse effects of drugs. Apart from demonstrable pathological sequelæ, the patient's vitality—the general level of health—may be lowered, and it is often possible by constitutional prescribing to raise the patient's vitality to the extent that functional disorders

and diseases with reversible pathological changes may be cured. It is sometimes also possible to help in this way to some extent even in incurable conditions.

ACUTE INFECTION

When a patient is slow in recovering from any acute infection certain remedies, such as *Sulphur*, *Psorium* and *Carbo veg.*, suggest themselves, but the prescription must be selected individually. In these circumstances or when prolonged ill-health follows, the nosodes of acute infection may be invaluable. Kent and others have rightly condemned the indiscriminate use of nosodes. The place of nosodes of acute infections is fairly clear. They should be considered (a) in the absence of satisfactory indications for a proved remedy and (b) when remedies apparently well chosen on a symptomatic basis fail to achieve adequate results.

Swan of America was the first to use nosodes extensively and among his followers may be included Allen, Compton Burnett and Tyler.

Dr. Tyler,³ writing on *Morbillinum*, said "Swan, pioneer in the use of disease products for the cure of like disease, received the inspiration and prepared from among many such substances, potentized measles, with which he did some astonishing work". "And now, after all these years of dullness and neglect, we are receiving fresh impetus, and are already getting sudden astonishing results. As a matter of fact we have for years been making play with *Variolinum*, *Tuberculinum*, *Lueticum*, *Medorrhinum*, *Influenzinum*: only, as said, *Morbillinum* and several others have till now not entered into the picture, and *Morbillinum* threatens to become the most important of the lot." "Everybody has had measles" and not everybody has managed to "annihilate the disease" so that nothing latent and threatening has remained. In future we shall do well to take notice when told of an old acute sickness, never, or very tardily, recovered from . . . , never well since diphtheria—scarlet fever—vaccination. "Tonsillitis followed by chorea, even rheumatism; heart damaged in childhood by rheumatic fever", these last put in a strong plea for that mighty remedy *Streptococcin*.

Having worked with Dr. Tyler for some years, I was able to observe her results at first hand, and they certainly were often remarkable. My impression is that her success was to some considerable extent due to careful constitutional prescribing before giving the nosode. Many of the patients had received her expert treatment even as long as years beforehand, but she did prescribe the appropriate nosode at the beginning if there were no obvious indications for a proved remedy, and the results then also were usually very good.

Apart from the probable advantage of careful constitutional prescribing prior to giving the appropriate nosode, there is another reason why these nosodes should be given only after due consideration. That is, that they may work like a charm or not at all. The same applies to administration of a potency of any drug which has adversely affected a patient.

Tyler published a number of her cases in *Homœopathy*. Here is one under the title "Rheumatism—*Scarlatina*". I have abbreviated the notes.⁴ "November 1939. Patient sent by her doctor with request for 'massage for her legs and feet for rheumatism'. She had had rheumatic pains for two years. Symptoms were indefinite as regards choice of remedy, but she had had scarlet fever as a child and it had left her with a 'bad ear'. R *Scarlatina* 200, three doses 6 hourly (Tyler's favourite prescription of the nosodes), no massage." In December 1939 there is a note: "Walking better. Pain gone from legs. Better in herself."

Scarlatina was repeated in February 1940 and again in October 1940 when Tyler wrote "The nosode was repeated as before. This was her second repeat in eleven months in which she has been practically well of two years' rheumatism. She has never needed or been given massage."

I had a case of this sort a few years ago, when no remedy seemed clearly indicated from the start. A University student had been unable to concentrate and suffered from attacks of vertigo when walking. He described this as feeling "as if he had put one foot in a small boat". This began after an attack of measles two years previously. The cause of the vertigo was obscure, he had been checked over at the E.N.T. department of a teaching school without a definite diagnosis having been made. *Morbillinum* 200, 1 dose, was followed by a short aggravation, then complete cure.

Apart from the nosodes of acute childish and other illnesses, it is worth while to note any tendency to streptococcal, staphylococcal or any bacterial or virus infection.

Tyler mentioned *Streptococcin* in relation to a rheumatic fever history, meaning that such a history would suggest *Streptococcin* should there be the indications for a nosode, as a constitutional remedy. Staphylococcal infection is common too, and *Staphylococcin* may be a useful remedy. A mongol child of eight years suffered from furunculosis of her back and perineum which was sluggish in responding to treatment. *Staphylococcin* 30 was followed by a rapid clearing of the condition. Several months later there was a recurrence and again *Staphylococcin* 30 cleared it up, and it is still clear after some months.

A lad of 15 years of age had suffered from eczema practically all his life, accompanied by secondary infection on many occasions. It was widespread, and he had been in hospital almost half his life, partly because of the severity of his condition and partly because home conditions were far from good. He responded slowly and unsatisfactorily to homœopathic treatment, till it was noted that he was always worse at night. A nightly aggravation is covered by about two hundred remedies, but when this remains an outstanding feature after apparently well indicated remedies have been given, *Lueticum* is nearly always required. He was given *Lueticum* 30, and this was followed by great improvement. Then a month later he developed widespread furunculosis. *Staphylococcin* 30 was given and there was a dramatic improvement. *Staphylococcin* had to be repeated six months later and he has been practically free from eczema and furunculosis for a year. It is, of course, impossible to assess the part played by *Staphylococcin* in this case, as *Lueticum* is associated with "successions of abscesses", a point also to remember in utilising the past history for prescribing. I mention this case as it illustrates the kind of case in which *Staphylococcin* seems to be indicated, that is to say, when there have been recurrent staphylococcal lesions over a long period of time and when staphylococcal lesions are widespread. At least it is worth a trial in these circumstances when all else fails. It would take a careful analysis of many cases to establish a "clinical proving" of *Staphylococcin*, which I believe would be of value in respect of many of the nosodes of acute infection.

A clinical proving should be based on a large number of cases, say at least 50. Before publishing the first paper on *Carcinosin*⁵ we had detailed notes of over 200 patients. Some years ago I made a brief study of *Streptococcin* from about thirty cases. The most definite symptom which emerged was "weepy, consolation aggravates" and this has been of value in confirming several subsequent prescriptions. Another symptom which emerged, although not quite so definitely,

was "better in the open air". Also *Streptococcin* has certain rheumatic symptoms indistinguishable from those of *Rhus tox.* and other remedies which one might expect from its pathological relationship to *Pyrogen*. "Worse in wet weather, worse on beginning to move, better for subsequent movement." A woman of 50 had fallen on her knees and had pain which had persisted for some months, although there was no evidence of a fracture or osteoarthritis. She had these symptoms, and *Rhus tox.* helped a little. *Arnica* and *Ruta* had no obvious effect. Many years previously she had suffered from quinsy and rheumatic fever. *Streptococcin* 30, three doses, 2 hourly, was followed by a dramatic and lasting freedom from pain, although she was not completely cured.

Along with these nosodes is a group of remedies with a pathological relationship to certain illnesses, including *Thuja*, *Drosera* and *Lathyrus sativa* in respect of vaccination, whooping cough and poliomyelitis. *Thuja* is well known for its importance in dealing with the after-effects of vaccination, immediate or remote, and it can be prescribed much in the same way as a nosode, that is to say, when there are no clear indications for another remedy, or when apparently indicated remedies do not act satisfactorily, even when symptomatic indications are absent. Failure to take after a first vaccination, and even very large vaccination scars, should also lead to consideration of *Thuja* in these circumstances. *Thuja* is a valuable remedy when there are no good indications for anything else in cases of acute bronchitis, slow to clear up, as pointed out by Clarke. If the patient has had a bad reaction to vaccination and/or a shiny face, these are excellent confirmatory symptoms.

Drosera is probably a better routine prescription for the aftermath of whooping cough than *Pertussin*. *Drosera* being a proved remedy may be symptomatically indicated, but often the choice is virtually impossible. *Pertussin* can be tried later if *Drosera* does not work.

A woman of 37 had suffered from migraine since childhood. The headaches were accompanied by an opening and shutting sensation in the head. After two years of prescribing she had fewer and less severe headaches. She was difficult, as there were too many "good prescribing symptoms". Then one day she complained of a cough which on enquiry was found to be paroxysmal and she had had bouts of paroxysmal coughing lasting a few days every year since whooping cough at the age of 8. She did not know whether or not whooping cough had preceded the migraine attacks, which maybe was not important in any case. *Drosera* 200, 1M and 10M was given. It had to be repeated six months later, since when she has been almost completely free from migraine for three years.

A woman of 38 complained of "rheumatism of the left shoulder". There was some wasting of the muscles of the shoulder girdle which followed poliomyelitis eight years previously. *Arsenicum alb.* 200, t.i.d. 2 days, b.d. 2 days, was prescribed. She was unduly weak, worse at midnight, she was chilly and had burning pains relieved by warmth. The treatment worked like a charm, but the pains returned immediately it was stopped. A second course was given with nearly as good a result, then a higher potency was given, but with not much effect. *Lathyrus sativa* 1M, three doses 12 hourly, was given and the pains cleared up for six months, when *Lathyrus sativa* 1M was again prescribed. She had no further pain for about 3½ years, when again *Lathyrus* was followed by relief. It does not seem to matter how long after a severe acute infection the appropriate remedy is given. Shortly after the war I was treating an elderly man for fibrositis of the lumbar region and troublesome paræsthesia of the

lower limbs. Despite the fact that careful investigation failed to reveal anything serious, he did not respond to treatment at all until he received *Typhoidinum* 200 on account of typhoid fever he had had in 1886. The following month he came up saying he felt very much better. Glandular fever may be very slow to clear up. *Carcinosin* may be a useful remedy for such cases.

Pyrogen is sometimes useful when there is a history of septicæmia, severe after-effects of dental extraction, or ill health commencing after an abortion in the absence of any obvious pelvic pathology. Enquiry should always be made, when possible, as to whether or not the mother suffered from any acute infection during pregnancy. The influence of German measles and probably other infections in causing malformation of the fœtus has been established, and it is therefore likely that less obvious after-effects occur in the same way that post-nasal illnesses may leave their mark.

CHRONIC INFECTION

Tuberculosis and venereal disease may call for the appropriate nosode, but fortunately these nosodes have been well proved and thoroughly clinically tested. It would be only in the absence of response to other apparently indicated remedies that they should be prescribed on the basis of the history alone.

The therapeutic field of nosodes of infection is vast and probably important, though not easy to exploit with our present limited knowledge. For example, brucellosis, with its protean manifestations, often missed because it is not considered as a diagnostic possibility, is regarded by some authorities as a widespread infection in this country. There is practically no reference to brucellosis in homœopathic literature at all.⁶

Nosodes tend to be neglected in homœopathic practice because of the influence of Hahnemann's theory of chronic disease and Kent's powerful endorsement of much of it.

INJURY

Arnica montana has been used domestically for centuries for the effects of falls and bruises, and its proving demonstrated that *Arnica* acts homœopathically. It was used by the Germans and the Russians to combat shock in battle casualties in the second world war. *Arnica* has been extensively used in homœopathic practice throughout the world for generations as the first remedy for the after-effects of injury. In any condition which has resulted from an injury, *Arnica* is worthy of consideration when other remedies fail, and is the remedy of choice when there are no special indications for another remedy.

For the after-effects of head injury there are a group of remedies, *Arnica*, *Cicuta*, *Hypericum*, *Nat. mur.*, *Nat. sulph.* and *Helleborus*, and these demand consideration also if there is a history of difficult birth. A child of six years of age took cold very frequently, had never thrived properly. He was markedly underweight, and had "no interest in life", very sensitive to noise, afraid of the noise of a high wind and had other symptoms of *Nat. sulph.* The history of a difficult birth confirmed the choice and the administration of *Nat. sulph.* was followed by a rapid improvement in health, mentally and physically.

Occasionally patients react in other ways after head injury. One child developed *Nux vomica* symptoms and was helped by *Nux vomica*. The list of such remedies is merely a record of frequent clinical associations.

Hypericum is the main remedy for spinal injuries. A woman of 34 developed ulcerative colitis which she attributed to a fall in which she hurt her back

nine months previously. Her constitutional remedy was *Sepia*, which she was given with some benefit, but not as much as one would expect. Higher potencies were given as the indications for *Sepia* seemed absolutely clear, but the results were not satisfactory. Then on the family history and the complementary relationship between *Sepia* and *Carcinosin*, a dose of *Carcinosin* 200 was given, but again without any dramatic effect. *Hypericum* 30, 200, 1M on consecutive days was given and a month later she said "I'm cured". There was a slight relapse two months later for which *Hypericum* was given, and she has been well for over a year. This is the kind of case which makes one feel that it is sometimes necessary to directly antidote some episode of the past.

Another woman of 45 suffered from ulcerative colitis for several years. I treated her for about three years with fair results, till she was given a high potency of *Arnica* on account of an old severe injury. She has been remarkably better during the past two years.

Everyone has been injured at some time or another; it is only when there has been an injury in relation to the onset of ill-health, or when there has been a severe injury in the past that it should call for consideration in constitutional prescribing.

PSYCHIC FACTORS

Sometimes there is a clear history of a complaint starting after grief, fear or other emotional upset. The lists of remedies clinically associated with such precipitating causes found in Kent's repertory and elsewhere may be valuable in giving confirmation to an individually chosen remedy. A boy of 15 years had suffered from enuresis since the age of 2½. He had been dry at 18 months but reverted to bedwetting when his mother was admitted to hospital. Many treatments had been given in vain before he came to Homœopathy. The boy was grossly overweight and had other symptoms suggesting *Graphites*, which is one of the remedies known to have the "effects of grief". He was given *Graphites* CM. There was an aggravation for a week or so then a remission for some months, when he had a slight recurrence. *Graphites* CM was given again, and he has been free from bedwetting for over a year. Of course, he might have "grown out of it" but the case illustrates the way in which knowledge of the precipitating cause may help to confirm the choice of a remedy.

Many years ago I saw an excellent example of the use of *Opium* for the effects of fright, or to be more accurate, the value of *Opium* for a patient who reacted to fright in the *Opium* way. A woman developed exophthalmic goitre which came on suddenly after a severe fright. Dr. Tyler elicited the characteristic *Opium* fear symptom which is that when the frightening incident is recalled, the patient experiences fear long after, maybe months or, in this case, years after. *Opium* CM cured her.

Very occasionally *Opium* is useful post-operatively when an old fright is presumably restimulated by another frightening experience. A boy of 11 years was extremely ill, for no obvious reason, after an operation designed to reconstruct a badly injured shoulder. On the strength of this possibility I gave *Opium* CM and the remedy had a very beneficial effect. A boy of six years developed a temperature of 103° F. after removal of tonsils and adenoids. A course of antibiotics was given by the house surgeon but the temperature was unaffected. I saw the boy, and prescribed *Sulphur* 200 on "absence of prescribing symptoms", again without result. Then on re-examining the boy in case something had been missed, my attention was drawn to a large burn scar on his

abdomen. There was nothing else to go on and I prescribed *Opium* CM. This was followed by a drop in temperature overnight. Afterwards I questioned the mother who said that the boy had screamed every time anyone touched his tummy from the time of the burn at 18 months till he was three years old—a typical *Opium* symptom. Quite often other remedies are of course required for after effects of fright. A girl of 9 years developed psoriasis in the form of small rounded areas less than a centimetre in diameter after a fright. *Opium* CM was prescribed over the telephone, but without any apparent benefit. When the mother brought the girl to see me it was obvious that in this case the remedy was *Aurum met.* which also has “the effects of fright”. It was given in the 200th potency. This was followed by an aggravation, then the psoriasis cleared up for a few months but returned. *Aurum met.* 10M was prescribed and the psoriasis disappeared and has not recurred during the past two years. A girl of 3½ years was admitted to hospital with a temperature of 105° F. in a drowsy delirious state. Her blood count showed 38,000 W.B.C., mainly polymorphs. The only positive finding on clinical examination was evidence of sinusitis accompanied by large tender cervical glands. The child was dangerously ill. Her face was hot and flushed. The pupils were pinpoint. She was muttering about “flies” although there were none in the ward, and she was obviously frightened but not from any apparent external cause. I ordered an antibiotic and *Opium* CM to be given hourly at first. That was 1.30 p.m. At 4.30 p.m. she was sitting on her mother’s lap watching television. The temperature had fallen to 102° F. and by 8.30 p.m. that evening the temperature was 98.4. There was a slight return of fever the following day which subsided next day, and the blood count came down dramatically. She made an uneventful recovery. The antibiotic had not been given, and was never needed. The cause of such a high temperature and much raised white blood count was never satisfactorily explained. On questioning the mother she said that her child had been frightened by a wasp three weeks before, which might conceivably have explained the “flies” but one cannot be sure. However, she was apparently “stuck in fright”, which is another way of describing the *Opium* fear reaction.

Sometimes an emotional upset may apparently occur prenatally. A girl of sixteen years of age was slow in recovering from influenza. Thinking back over the twelve years I had looked after her, this was the usual pattern; there was sluggishness in recovering from every acute illness for no obvious reason. Going back over the history I discovered that the mother had been frightened by a doctor during pregnancy on being told that she might lose her baby. She said “I felt the shock go right through me”. The girl was given *Opium* CM and made a quicker recovery, but not only that, she became physically more robust and her ability to study increased quite definitely.

Staphisagria is well known for its effects when there is a strong sense of injustice. A number of remedies have this—listed in Kent’s repertory under “Mortification”—but *Staphisagria* is most often needed when this is a leading symptom, in my experience.

As a rule children greatly prefer to be in the ward with other children rather than in a cubicle. One little boy, however, objected strongly when he was admitted to the general ward. Everything was wrong, and he insisted he be put back in his cubicle. The ward sister detected a strong sense of injustice, and gave him a dose of *Staphisagria* 200. She told me that within ten minutes he was delighted to be in the ward and everything was fine! Sometimes there is a prolonged feeling of injustice. Dr. Tyler quoted a very typical case⁷ of an

army officer who had been passed over as regards promotion during the first world war and came back into civilian life generally unwell and disgruntled. He was given *Staphisagria* in high potency with excellent results.

Causticum is sometimes indicated after acute fright, or prolonged anxiety. In 1939 a man of 52 came to the hospital suffering from loss of voice. He was distressed because he was foreman of a factory which was closing down and he had to give notice to his men, some well on in years, who might find difficulty in getting other work. *Causticum* is worth trying for loss of voice whether from psychological or inflammatory causes, when there is little on which to prescribe accurately and, of course, it covered his sympathetic nature. The interesting thing about this man was that he had been buried alive during the first world war and when rescued, it was discovered that his hair had fallen out. He had been almost completely bald since 1916. He was given *Causticum* 200 and his voice returned soon after. About two months later he said that his wife and daughter had noticed that his hair was growing in. A fine downy fluff, about a quarter of an inch in depth, covered the bald patch. I saw him after the war and there was no further growth of hair even with higher potencies of *Causticum*, and other remedies. The remarkable feature was that *Causticum* apparently had caused such an effect after twenty-three years.

DRUGS

Hahnemann observed that *Nux vomica* was a useful remedy for "the evil consequences of coffee and wine drinking" and since then *Nux vomica* has been extensively used for the after effects of drugs in general. *Pulsatilla* has been found effective in treating patients upset by iron. *Nat. mur.* and other remedies have been used to counteract the effects of prolonged drugging with quinine in malarious districts.

Thus certain relationships have been established between constitution and the influence of drugs. As a rule constitutional treatment can deal with the after-effects of drugs once the drug has been excreted. There are also many records of drug effects being antidoted by the same drug given in potency. It is better to prescribe constitutionally first, as a potency of the drug may work dramatically or not at all. It is interesting that the same applies to the nosodes of acute infection in respect of ill health following an acute illness as already mentioned.

Dr. Charles C. Bowes related a case⁸ in which a potency of the chemical responsible for the illness was successful in curing. "A child of four years falling into short spells of unconsciousness, unable to control the flow of urine day or night, was absolutely cured by a dose of *Terebintha* 1M. The history of the case was that the child had drunk a lot of turpentine when 18 months old and had gone from bad to worse ever since. She never had a fit after that dose, and gradually but quickly got over the enuresis."

Some years ago I treated a child suffering from an almost purely allergic type of asthma. The attacks occurred about once a week on an average. The first attack had been apparently caused by inhaling sulphur fumes. In this case the constitutional remedy was *Sulphur*, which was given in the 30th, later the 200th, potency at long intervals, repeating when there was a relapse. The child was completely cured.

A woman of 42 developed psoriasis consisting of rounded areas about 2 centimetres in diameter, widely distributed over her trunk and limbs, after treatment for tonsillitis with sulphapyridine. *Arsenicum alb.* was prescribed

but without any effect and a month later she was given *Sulphapyridine* 30. The psoriasis cleared up completely and had not returned six years later. Many cases of this kind have been reported in the homœopathic literature in respect of a wide range of chemicals.

Sometimes the child may be affected by drugs taken by the mother. In another case previously recorded, a boy of 14 years of age was said by his teachers to be intelligent, but somehow "could not use his brain". His mother had been taking *Pethidine* during pregnancy on account of osteomyelitis. The boy was given *Pethidine* 30 and his ability to study shot up.

It is interesting to speculate on what exactly happens in such cases when the drug which has adversely influenced a patient helps dramatically when given in potency. Is it that in these successful cases the drug is in fact one of the constitutional remedies for that individual, that it would have helped in any case whether the patient had had an overdose of it or not?

For some years now I have taken the "anæsthetic history" of every patient and this is occasionally very rewarding in constitutional prescribing.⁹ The most definite feature which has emerged is that patients suffering from liver or gall bladder disease who have had a bad reaction to chloroform, usually benefit from chloroform in potency. Such a history is often obtainable from older patients. It is probable that this just means that chloroform, with its known toxic effect on the liver, may be a useful "pathological" liver remedy. An infant of six months old kept her parents awake through flatulence although there was no obvious feeding mismanagement. *Lycopodium* and *Raphanus* were helpful, but the parents were getting worn out through lack of sleep despite this. The child had been born by cæsarean section in 1961, and it is very unlikely that any chloroform was given. As there was a possibility of some very slight liver damage I tried *Chloroform* 30, and there has been no trouble for the past nine months.

A man of 45 who had suffered from very severe attacks of asthma for ten years responded to some extent to constitutional treatment. He had been born under chloroform anæsthesia, and his mother had been greatly upset by the anæsthetic. *Nat. sulph.* and *Lachesis*, both liver remedies, had seemed to help him. He was given *Chloroform* 30, and later 200, and he has been practically free from asthma for over three years.

A woman of twenty complained of bouts of colicky epigastric pains which began at the age of eight. The attacks had become much more frequent and more severe during the previous year. Investigations were all negative. *Colocynth* 3x was found to be an effective remedy in relieving the pains, but despite careful constitutional treatment the number of attacks, about one in two weeks, was not reduced. There was a strong family history of cancer and four siblings had been helped by *Carcinosin*. *Carcinosin adenostum* 30 was given, but without apparent effect. She was the only one of the family born under chloroform anæsthesia and on this fact, together with symptoms of liver dysfunction, *Chloroform* 30 was tried. The attacks cleared for three months and then returned, but not so severely. *Chloroform* 100 was given and followed by practically complete freedom for over three years. She became pregnant, and during pregnancy the colicky pains returned quite severely. She came to see me in July 1962 and *Chloroform* 200 wiped out the pains.

OTHER FACTORS

A number of items may be of value in some cases. Exposure to radiation if

judged to be excessive may call for a trial of *X-ray* 30 or 200 or *Radium bromide* 30 or 200 as intercurrent remedies. Illnesses beginning at puberty suggest a careful look at *Pulsatilla* and when ill-health starts after the menopause, *Lachesis* and others.

FAMILY HISTORY

In making a psychomatic homœopathic prescription or a prescription based on the individual's past history, the patient is regarded against a background of a theoretical norm. The same applies to "family history" and the same rule regarding any uncertain symptoms—if in doubt discard them and look for something definite. Should there be a strong history of tuberculosis or cancer in the family, the appropriate nosodes deserve consideration. Recent studies of *Carsinosin* suggest that it may be a useful remedy when one or more of a group of diseases are strongly represented in the family history, carcinoma, leukæmia, tuberculosis and diabetes.

IN CONCLUSION

Richard Hughes said "The rule *Similia Similibus* can obviously be carried out only in proportion as the effects of drugs on the healthy body are ascertained". His book, *The Principles and Practice of Homœopathy* is a magnificent contribution to sound pathological prescribing, a knowledge of which is fundamental to homœopathic practice. Similar remedies can, however, be chosen otherwise. There are different bases of similarity and there is no reason why different approaches to homœopathic prescribing should not be appropriately utilized.

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